

Table S2: Relative risks for the effects of alcohol use on disease outcomes from meta-analyses.

<u>Disease outcome</u>	<u>Sex</u>	<u>Age (years)</u>	<u>I^a</u>	<u>II</u>	<u>III</u>	<u>IV</u>
<u>Ischemic heart disease [1-3]</u>	<u>Both</u>	<u>30–44</u>	<u>1.00</u>	<u>0.60</u>	<u>0.62</u>	<u>1.00^b</u>
		<u>45–50</u>	<u>1.00</u>	<u>0.63</u>	<u>0.65</u>	<u>1.00^b</u>
		<u>60–69</u>	<u>1.00</u>	<u>0.82</u>	<u>0.83</u>	<u>1.00^b</u>
		<u>70–79</u>	<u>1.00</u>	<u>0.92</u>	<u>0.93</u>	<u>1.00^b</u>
		<u>≥80</u>	<u>1.00</u>	<u>0.97</u>	<u>0.98</u>	<u>1.00^b</u>
<u>Ischemic stroke [2-4]</u>	<u>Men</u>	<u>30–44</u>	<u>1.00</u>	<u>0.83^b</u>	<u>1.07^b</u>	<u>3.84</u>
		<u>45–50</u>	<u>1.00</u>	<u>0.88^b</u>	<u>1.05^b</u>	<u>2.52</u>
		<u>60–69</u>	<u>1.00</u>	<u>0.94^b</u>	<u>1.02^b</u>	<u>1.69</u>
		<u>70–79</u>	<u>1.00</u>	<u>0.97^b</u>	<u>1.01^b</u>	<u>1.32</u>
		<u>≥80</u>	<u>1.00</u>	<u>1.00^b</u>	<u>1.00^b</u>	<u>1.00^b</u>
	<u>Women</u>	<u>30–44</u>	<u>1.00</u>	<u>0.88^b</u>	<u>0.85^b</u>	<u>1.33^b</u>
		<u>45–50</u>	<u>1.00</u>	<u>0.91^b</u>	<u>0.89^b</u>	<u>1.22^b</u>
		<u>60–69</u>	<u>1.00</u>	<u>0.96^b</u>	<u>0.95^b</u>	<u>1.10^b</u>
		<u>70–79</u>	<u>1.00</u>	<u>0.98^b</u>	<u>0.98^b</u>	<u>1.05^b</u>
		<u>≥80</u>	<u>1.00</u>	<u>1.00^b</u>	<u>1.00^b</u>	<u>1.00^b</u>
<u>Hemorrhagic stroke [2-4]</u>	<u>Men</u>	<u>30–44</u>	<u>1.00</u>	<u>1.65^b</u>	<u>3.16^b</u>	<u>6.65</u>
		<u>45–50</u>	<u>1.00</u>	<u>1.42^b</u>	<u>2.22^b</u>	<u>3.60</u>
		<u>60–69</u>	<u>1.00</u>	<u>1.19^b</u>	<u>1.55^b</u>	<u>2.18</u>
		<u>70–79</u>	<u>1.00</u>	<u>1.09^b</u>	<u>1.26^b</u>	<u>1.55</u>
		<u>≥80</u>	<u>1.00</u>	<u>1.00^b</u>	<u>1.00^b</u>	<u>1.00^b</u>
	<u>Women</u>	<u>30–44</u>	<u>1.00</u>	<u>1.30^b</u>	<u>2.07^b</u>	<u>3.89^b</u>
		<u>45–50</u>	<u>1.00</u>	<u>1.20^b</u>	<u>1.67^b</u>	<u>2.54^b</u>
		<u>60–69</u>	<u>1.00</u>	<u>1.09^b</u>	<u>1.30^b</u>	<u>1.70^b</u>
		<u>70–79</u>	<u>1.00</u>	<u>1.04^b</u>	<u>1.14^b</u>	<u>1.32^b</u>
		<u>≥80</u>	<u>1.00</u>	<u>1.00^b</u>	<u>1.00^b</u>	<u>1.00^b</u>
<u>Hypertensive disease [2,5]</u>	<u>Men</u>	<u>30≥</u>	<u>1.00</u>	<u>1.40</u>	<u>2.00</u>	<u>4.10</u>
	<u>Women</u>	<u>30≥</u>	<u>1.00</u>	<u>1.40</u>	<u>2.00</u>	<u>2.00</u>
<u>Cardiac arrhythmias [2,6]</u>	<u>Both</u>	<u>30≥</u>	<u>1.00</u>	<u>1.51</u>	<u>2.23</u>	<u>2.23</u>
<u>Larynx cancer [2,6]</u>	<u>Both</u>	<u>30≥</u>	<u>1.00</u>	<u>1.83</u>	<u>3.90</u>	<u>4.93</u>
<u>Mouth and pharynx cancer [2]</u>	<u>Both</u>	<u>30≥</u>	<u>1.00</u>	<u>1.45</u>	<u>1.85</u>	<u>5.39</u>
<u>Selected other cancers [3]</u>	<u>Both</u>	<u>30≥</u>	<u>1.00</u>	<u>1.10</u>	<u>1.30</u>	<u>1.70</u>
<u>Diabetes mellitus [2]</u>	<u>Men</u>	<u>30≥</u>	<u>1.00</u>	<u>0.99</u>	<u>0.57</u>	<u>0.73</u>
	<u>Women</u>	<u>30≥</u>	<u>1.00</u>	<u>0.92</u>	<u>0.87</u>	<u>1.13</u>
<u>Liver cirrhosis [3,6]</u>	<u>Both</u>	<u>30≥</u>	<u>1.00</u>	<u>1.30</u>	<u>9.50</u>	<u>13.00</u>
<u>Pancreatitis [2,5]</u>	<u>Men</u>	<u>30≥</u>	<u>1.00</u>	<u>1.30</u>	<u>1.80</u>	<u>3.20</u>
	<u>Women</u>	<u>30≥</u>	<u>1.00</u>	<u>1.30</u>	<u>1.80</u>	<u>1.80</u>

I, Abstainers; II, <40 g/day of ethanol intake; III, 40–59.9 g/day; IV, >60 g/day.

^a Reference category

^b We replaced these statistically insignificant relative risks with 1 in our analysis.

References

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~~Gender and resistance in Kashmir: old paradigms, new approaches¹~~

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In July 2016, violence erupted in Srinagar and other towns in the Kashmir Valley across the northern Indian state of Jammu and Kashmir. This violence came in response to the killing of a popular young militant, Burhan Wani, by the Indian security forces. Tens of thousands of people entered the streets for Wani's funeral, the processions instantly morphing into protests against India. The demonstrations were met with a heavy-handed response by Indian troops, and Kashmir was brought to a standstill for months by curfews and strikes. Between July and October 2016, more than 90 people were reported killed and thousands were injured by the rubber bullets used by the Indian army to control the masses (Waheed 2016). As of August 2018, when we finalized this Conversation section, the protests have subsided but solution to the conflict nowhere in sight.

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This violence heralded a new phase of the protracted mobilization for *azaadi* (freedom) in Kashmir, which can be traced to the early twentieth century. The Kashmir region is located in the most northern part of South Asia, roughly encompassing the former Princely State of Jammu and Kashmir that was ruled by the Dogra Dynasty between 1846 and 1952. Despite a strong indigenous mobilization demanding democracy, political rights and redistribution of land and resources, the Princely State ended up as part of both India and Pakistan following the first Indo-Pak war in 1947–1948. India and Pakistan base their territorial claims on Kashmir as an integral part of their national identities and have been engaged in three major wars and minor border skirmishes since 1948.

In 1987, people took to the street *en masse* to protest the defeat of a popular local party in allegedly rigged state elections. This ignited the contemporary Kashmiri movement for self-determination. Thousands of men and some women joined militant resistance groups in a guerrilla war against the Indian state and security forces. The Indian authorities responded militarily, sending in an unprecedented number of troops and using torture, disappearances, and sexual violence to curb the resistance movement. As the army viewed Kashmiri men as suspicious and potential militants, women began helping with smuggling weapons and clearing civilians ahead of attacks, in addition to providing shelter, food and medical care to militants and their families (Parashar 2014). While the popularity of the militants waned in the mid-1990s, anger and frustration about the political situation continued to simmer as the demands for self-determination were not yet met. The summers of 2008, 2009 and 2010 saw large-scale clashes between the Indian army and protestors. However, the mass mobilization of summer 2016 marks a different turn. This new phase is led by young people who grew up during the militancy years of the 1990s. It can be characterized by mass demonstrations on the streets of cities and towns all over the Kashmir Valley, as well as *hartals* (strikes) observed by government employees, school teachers and many others.

This Conversations forum is a timely intervention to explore how a gender analysis contributes to our understanding of the ongoing conflict in Kashmir. It specifically focuses on the different conceptions of femininities and masculinities held by Kashmiris and the Indian state. A growing body of feminist literature has explored the impact of the long-term conflict on Kashmiri women (Manchanda 2001; Kazi 2009), unpacked the roles women played in the resistance movement (Kazi 2009; Parashar 2014) and uncovered their stake in peacebuilding (Shekhawat 2014). Yet, there is a gap in the

scholarship on the role of gender norms and practices in driving the resistance movement and the responses from the Indian state and security forces. Since the late 1980s, gendered images of victimized, mourning women and gun-toting, violent men have frequently circulated, leaving little space for non-essentialized femininities and masculinities in representations on Kashmir. We therefore invited perspectives that consider manifestations of femininities and masculinities in more nuanced ways. The pieces in this forum use both theoretical and empirical approaches to unpack how women and men have engaged in the resistance movement, tracing how femininities and masculinities map onto gendered bodies in a context shaped by military occupation. In the spirit of feminist activism and politics, this dialogue between the participants seeks to challenge established paradigms and open new approaches to the study of the conflict in Kashmir.

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